INVESTMENT THESIS

Validate the oncology ISV wedge—do not launch broadly

Win healthcare software developers through source-backed specialty workflows, then monetize repeat API usage.

MARKET

Where to play

Clinical workflow ISVs; oncology is the first use case

PRODUCT

What to build

Developer quickstart + evaluation + spend transparency

ECONOMICS

How to monetize

Usage-based API billings → sustained use → commitments

CHANNEL

How to distribute

Direct API first; cloud and healthcare partners as needed

DECISION REQUESTED

Approve bounded research, evaluation, and design-partner validation—no scale commitment yet.

WHERE TO COMPETE

Clinical workflow ISVs are the first market

Weighted analyst scores from M6.1; these are research judgments, not observed demand.



Key distinction: oncology is the initial specialty workflow within clinical-workflow ISVs—not a sixth competing market.

CUSTOMER WEDGE

Start with recurring, evidence-grounded oncology workflows

Avoid generic ambient notes; evaluate buyer problems where longitudinal evidence and traceability matter.

FIRST WORKFLOW

Source-backed oncology synthesis

Longitudinal chart context, contradictions, citations, clinician review and handoffs

RESEARCH ONLY

Six discovery candidates

Publicly researched targets, not signed design partners or known Claude customers

Navigating Care	Triage	**Canopy**	Monitoring	**OncoHealth**	UM review
Candid Health	Denials	**Tennr**	Referrals	**Stedi**	Payer APIs

Research plan: validate workflow frequency, existing model choice, buyer authority, and willingness to pay before prioritizing accounts.

PRODUCT STRATEGY

Make the first evaluated workflow easy to reach

North star: external healthcare ISVs with sustained, paid production API usage—not signups.

01 Discover	02 Activate	03 Evaluate	04 Prepare	05 Produce	06 Expand
Docs	First call	Evidence	Controls	Live use	Repeat

P0 INVESTMENTS — FIVE CAPABILITIES

Quickstart	Evaluation	Cost model	Readiness	Telemetry
FHIR synthetic example	Evidence + review	Per accepted workflow	PHI / security gates	Cohort conversion

Measure activation → evaluated pilot → paid production at Day 180; proposed targets are unvalidated, cohort-specific hypotheses.

COMPETITION AND EVIDENCE

Prove Claude value with a matched, blinded evaluation

No current-model clinical-quality winner has been established by this case study.

CANDIDATE 1

Claude

Citations and developer platform; verify structured-output workaround

CANDIDATE 2

GPT

Competing models/tools; compare safety, evidence and complete cost

CANDIDATE 3

Gemini

Multimodal/cloud choices; compare deployment and quality

EVALUATION DESIGN

Evidence	Supported claims, citation validity, conflicts
Clinical risk	Harmful errors, abstentions and review
Workflow value	Accepted-case rate and reviewer time
Economics	Latency and cost per accepted workflow

Compare the same synthetic/deidentified cases and a no-AI baseline. Clinician review is proposed; none has occurred in M6.

MONETIZATION ECONOMICS

Consumption intensity matters more than ISV count

M6.2 scenarios are illustrative API billings—not recognized Anthropic revenue or margins.

$125K

First-year base billings

Modeled 12-month ramp

$269K

Month-12 annualized

50 paying ISVs

186

ISVs for $1M annualized

At 30k monthly workflows each

DOWNSTREAM ISV UNIT ECONOMICS

30,000 modeled workflows / month **vs.** **31,558** estimated break-even workflows

Risk: customer workflow frequency, clinician review costs, discounts and actual paid conversion remain unverified.

RISK-ADJUSTED BUSINESS CASE

The downside justifies validation before scale

Modeled Year-1 API billings; hypothetical scenarios are not a financial forecast.



Do not infer platform profit

Model-provider inference cost, selling expense, support and real contracts are unavailable. Weights are subjective.

COMMERCIAL ROUTE AND TRUST

Use direct API first; open channels when buyer needs justify them

Distribution choice, HIPAA eligibility and partner economics vary by service and contractual path.

1 **Direct API**	2 **Cloud channels**	3 **Services partners**	4 **Oncology platforms**
Developer evaluation	Procurement fit	Complex integrations	Later distribution

THREE NO-GO GATES BEFORE ANY PHI DEPLOYMENT

01 Agreement + features	**02 Data-flow approval**	**03 Clinical intended use**
Executed BAA; eligible API configuration and tools	Prompts, logs, subprocessors, vendor/security review	Safety assessment, clinician sign-off, regulatory scope

ChartGround M1–M5 passed local tests; this is not PHI authorization, production certification or real financial settlement.

VALIDATION ROADMAP

A 90-day decision; a 180-day production assessment

These are proposed external customer-validation activities—not work already completed or customer commitments.

D0–30

Discover

6–8 real buyer / developer interviews; ≥2 repeated costly problems

D31–60

Evaluate

Matched models, human review, workflow frequency and cost

D61–90

Decide

Sponsor, readiness, economic evidence; go / iterate / stop

D90–180

Observe

Paid production in mature cohorts and sustained non-test usage

Current evidence status

20 accounts researched • 0 customer interviews • 0 external design partners • 0 paid production customers

EXECUTIVE INVESTMENT DECISION

Authorize a bounded validation effort—not a broad launch

An independently developed proposal. Spending, prospective accounts and model performance require external validation.

GO when	Independent sponsor, repeat workflow, safe model quality, clear cost-per-accepted-case
ITERATE when	Developer interest exists but evaluation, security, or economics remain uncertain
STOP / PIVOT when	No differentiated demand, unacceptable clinical risk, or unfavorable sustainable economics

Executive takeaway: one targeted market, explicit unit economics, testable model quality, and defined stop rules.